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Comments of Public Health Innovators, LLC

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Public Health Innovators, LLC respectfully submits comments in response to the Request for Information to strengthen the digital health product market. Public Health Innovators appreciates the Agency's interest in public input and applauds efforts to expand patient access to data and promote technologies that improve health outcomes, enhance quality, and reduce provider burnout and healthcare costs.

Since 2016, Public Health Innovators, LLC, has bridged digital inclusion and healthcare to ensure that all populations can benefit from digital health. As its Principal, I have over two decades of experience in clinical and health services research and digital medicine, with a career-long focus on populations living in conditions that undermine health.

CMS is uniquely positioned to ensure that digital health innovation benefits all patients through digitally inclusive efforts. The National Digital Inclusion Alliance defines digital inclusion as "the activities necessary to ensure that all individuals and communities, including the most disadvantaged, have access to and use of information and communication technologies (ICTs)." These include:

1) affordable, robust broadband internet service;
2) internet-enabled devices that meet the needs of the user;
3) access to digital literacy training;
4) quality technical support; and
5) applications and online content designed to enable and encourage self-sufficiency, participation, and collaboration.

The 2021 Digital Equity Act (DEA) identified "covered population" groups that face specific barriers to using the internet, including:

- Rural residents lacking high-speed broadband;
- Older adults with limited digital skills and heightened concerns about fraud;
- People[1] with physical disabilities that hinder device use;
- Individuals with language barriers;
- Low-income households unable to afford devices or internet;
- Formerly incarcerated individuals unfamiliar with digital tools;
- Veterans unable to access healthcare; and
- Racial and ethnic minorities affected by numerous structural and institutionalized barriers to internet access and digital literacy.

[1] Sarah S Nouri et al., "Patient Characteristics Associated with Objective Measures of Digital Health Tool Use in the United States: A Literature Review,"*: JAMIA* 27, no. 5 (May 2, 2020): 834–41, https://doi.org/10.1093/jamia/ocaa024.

Research[1] clearly associates these characteristics with use of digital health technology.

The DEA legislation provides CMS with a framework for supporting digital health technology adoption using strategies that are tailored for structural and systemic barriers facing different populations. For example, older adults learn technology differently than younger people. Due to the overlap of historic economic and recent digital redlining, Black and Hispanic people, and members of low-income households have had reduced access to affordable internet, limiting their opportunities to build digital skills and confidence.[2] Such barriers contribute to lower use of patient portals and video telehealth among individuals who are Black, non-English-speaking, disabled, or older.[3]

Every U.S. state developed federally approved **plans to expand access to affordable broadband internet[4]** through the Infrastructure Investment and Jobs Act's Broadband Equity, Access, and Deployment (BEAD) program.[5] However, a June 6, 2025 Restructuring Policy Notice[6] imposed changes that have delayed progress and may limit household access to reliable, affordable internet. Every state was also poised to implement federally approved **Digital Equity plans[7] to increase internet adoption and meaningful use** with state Capacity Grants and over $700,000,000 in Competitive Grants that had been recommended for funding.[8] All proposed projects addressed health outcomes, and at least twenty of the recommended grants focused on healthcare. On May 9,2025, the Administration canceled the entire $2.7 billion DEA program. [9] [10]

CMS should take intentional steps to help the health sector leverage its unmatched potential for identifying and addressing digital access and skill disparities by supporting digital health adoption. Key steps include:

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[2] Amy Sheon and Elaine Khoong, "Pillars, Policies, & Plausible Pathways Linking Digital Inclusion & Health Equity," *Health Affairs*, Invited Health Policy Brief, (May 2, 2024), https://www.healthaffairs.org/content/briefs/ha-pillars-policies-and-plausible-pathways-linking-digital-inclusion-and-health-equity.

[3] Sarah Nouri et al., "Addressing Equity in Telemedicine for Chronic Disease Management During the Covid-19 Pandemic," *NEJM Catalyst Innovations in Care Delivery*, May 4, 2020, https://catalyst.nejm.org/doi/full/10.1056/CAT.20.0123.

[4] "Every State and Territory Is Ready to Implement Internet for All, NTIA, https://www.ntia.gov/blog/2024/every-state-and-territory-ready-implement-internet-all.

[5] "Broadband Equity Access and Deployment Program | BroadbandUSA," https://broadbandusa.ntia.gov/funding-programs/broadband-equity-access-and-deployment-bead-program.

[6] "BEAD Restructuring Policy Notice | BroadbandUSA," https://broadbandusa.ntia.gov/funding-programs/policies-waivers/BEAD_Restructuring_Policy_Notice.

[7] "What's Your State's Digital Equity Plan?" Benton Foundation, February 12, 2024, https://www.benton.org/blog/whats-your-states-digital-equity-plan.

[8] "Biden-Harris Administration Approves and Recommends for Award Digital Equity Capacity Grant Applications Totaling More Than $85.1 Million | Ncbroadband.Gov," https://www.ncbroadband.gov/news/press-releases/2024/12/13/biden-harris-administration-approves-and-recommends-award-digital-equity-capacity-grant-applications.

[9] Donald J. Trump, "I Am Ending Digital Equity Program Immediately," Social media platform, *I Am Ending Digital Equity Program Immediately* (blog), May 8, 2025, https://truthsocial.com/@realDonaldTrump/posts/114474136573150113.

[10] "NTIA Terminates Digital Equity Act Grants | National Association of Counties," https://www.naco.org/news/ntia-terminates-digital-equity-act-grants.

- Encourage the Administration to restore BEAD and DEA funding.
- Collaborate with the Commerce Department so that federal broadband efforts benefit Americans' heath.
- Recognize that technology alone is insufficient. Support the "Human Infrastructure of Broadband"[11] – trusted individuals helping people connect and use technology.
- Train existing healthcare personnel to perform digital navigation services or train experienced digital navigators to work in and with the health sector.
- Recognize digital navigators as reimbursable members of healthcare teams.
- Recognize digital navigation as reimbursable healthcare service.
- Encourage development of validated instruments to screen patients for digital access and digital health literacy.
- Develop standard terminology for digital access/health literacy assessment, goal setting, referral, and services to support interoperable data exchange.
- Remove regulatory barriers preventing distribution of restricted devices needed for using connected devices such as continuous glucose monitors.

These actions are necessary for digital health to improve health outcomes, care quality, and cost efficiency.

Responses to specific questions follow:

[11] Bryne and Prasad, "The Human Infrastructure of Broadband: Looking Back, Looking Around and Looking Ahead." The Benton Institute for Broadband and Society, January 2025, https://www.benton.org/sites/default/files/Human.pdf.

Obtain input from individuals with limited digital access and skills [PC-1, PC-5, TD-1]

CMS should limit payment for digital health technology unless assured that people who face systemic barriers to digital health adoption have been involved at every stage of product development and product deployment.

For this to happen, I suggest that CMS partner with the National Digital Inclusion Alliance (NDIA) to convene stakeholders and train them as advisors to developers, funders, payers, and providers.

Excellent resources are available to support this work. [12]

Products useful for Medicare beneficiaries [PC-3]

Caris Health offers cellular-enabled tablets that make it easy for patients with low digital skills and limited internet access to use portals and telehealth services.

A product like this could assist family members to communicate with caregivers working with their loved ones.

Missing features [PC-4, PC-6]

The mobile version of patient portals should graph lab results, as currently available in the desktop version.

To make digital health products accessible and easy to use for Medicare beneficiaries and caregivers, particularly those with limited prior experience using digital tools and services, all digital health tools should have language translation and accessibility features.

They should function on low end Android and IOS devices and be backward compatible with devices up to six years old. They should come with training modules that have been developed with input from digital inclusion experts.

Methods of collecting real-world data on digital health products' impact on health outcomes [PC-7]

Health systems already track every click in the patient portal as well as patient interactions with tech support. This information should be mined to show how distinct groups utilize technology and its impact. Subgroup analysis should be conducted for all "covered population" groups defined in the Digital Equity Act. In addition, differences should be examined by insurance type and geography (urban, rural, suburban), as well as by historic economic redlining classification status, and contemporary segregation by race and poverty.

Data valuable to patients and caregivers [PC-8]

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[12] Rodriguez, Clark, and Bates, "Digital Health Equity as a Necessity in the 21st Century Cures Act Era"; Rodriguez, Shachar, and Bates, "Digital Inclusion as Health Care — Supporting Health Care Equity with Digital-Infrastructure Initiatives"; Rodriguez et al., "Digital Healthcare Equity in Primary Care"; Voorheis et al., "Consolidated Principles for Equitable and Inclusive Digital Health and Virtual Care Co-Design"; Henni et al., "The Experiences, Needs and Barriers of People with Impairments Related to Usability and Accessibility of Digital Health Solutions, Levels of Involvement in the Design Process and Strategies for Participatory and Universal Design"; "A Guide to Co-Creating with People with Lived Experience"; Agency for Healthcare Research and Quality, "A Practical Guide for Implementing the Digital Healthcare Equity Framework."

CGM data are immensely valuable, but patients need to be able to download their data in granular form, e.g., glucose by minute, with easy tools for analyzing associations with activity and dietary intake.

Obstacles to provider use of digital health services [PR-1a]

First, the Stark Law and anti-kickback provisions should be reformed to allow health systems or health plans to distribute devices needed to run prescribed technology. The distributed devices should NOT be restricted to health care uses. No patient wants to carry a second smartphone just to monitor their glucose.

Second, health systems and health plans should employ, and insurance should reimburse the cost of DIGITAL NAVIGATORS–people who are trained to help patients access devices, broadband, and foundational digital skills (e.g., internet use, device navigation, app downloads, online safety, login, and authentication. These individuals are trusted by those they assist, often because they have a shared lived experience. Then, DIGITAL HEALTH NAVIGATORS should be reimbursed to help people obtain and use the specific devices and applications needed to access digital health technology, and to use the technology itself. Both types of navigators should understand technology-related learning needs specific to different populations and individuals.

Third, technical support staff, whether attached to providers or technology manufacturers/distributors, need to be aware of and able to address the foundational digital skills underlying many requests for assistance, as well as the challenges arising from use of older, lower cost devices and poor quality internet service. They should be prepared to refer patients to in-house or community digital navigators for assistance.

Information providers should share with patients about digital products [PR-1b]

Patients need to know who will have access to what data, and whether applications require information such as sensitive such as location, microphone, and camera data. Many resources can be found in the Digital Medicine Society's Toolkits for Inclusive Development[13] and Deployment[14] of Digital Health Technology.

Obstacles that prevent development, deployment, or effective utilization [PR-2]

Patients must have devices, connectivity, and basic digital skills to have confidence to use digital health, and providers should be prepared to explain why and how products can be used.

Since 1/1/2024, CMS has required Medicare Advantage (MA) plans to screen for digital access and digital health literacy gaps and provide digital health literacy education. CMS has provided minimal guidance and included minimal reporting requirements. CMS should evaluate implementation and outcomes and convene digital inclusion experts to provide guidance on implementation and reporting. Improving the ability of all populations to use digital health will improve the market for digital health technology.

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[13] Digital Medicine Society, "Toolkit for the Inclusive Development of Digital Health Technology." https://datacc.dimesociety.org/development/

[14] Digital Medicine Society, "Toolkit for the Inclusive Deployment of Digital Health Technology." https://datacc.dimesociety.org/deployment/

The patient portal is a profoundly useful technology because it is a single application that can be tailored for each organization and offers resources specific to every health condition. The messaging feature seems to have taken healthcare by surprise, contributing to overwork and burnout. Messaging should automate triaging so that each response comes from an individual working at the top of their license. Time for response to messaging should be factored into provider scheduling and reimbursement to encourage, rather than discourage messaging. At the same time, patients require guidance to craft concise, appropriate, and targeted messages.

Incentives to encourage APMs…[VB-1]

Foundational digital access and literacy gaps, which can be above 50% in certain populations and geographies, are the biggest barriers to adoption. This costs APMs by limiting the number of users and the impact of the technology.

To mitigate this, CMS needs to devote resources to helping health systems and plans screen patients for digital health access and literacy barriers and helping them address these gaps. This includes:

- Support development of screening instruments. CMS spent over a decade developing social needs screening instruments and processes. The process should be much faster now but needs coordination and prioritization. Experts who can assist with this process can be found in the National Digital Inclusion Alliance's Digital Equity Research Inventory: https://www.digitalinclusion.org/resource/digital-equity-researcher-inventory.
- Require digital health access and literacy screening and education. Consider adding digital health access and literacy screening to existing social needs screening.
- Encourage use of social need closed loop referral systems for digital needs. This requires development of a taxonomy and populating community resource databases with digital inclusion organizations.
- Support these community organizations to provide foundational digital access and literacy support.
- Support the Gravity Project to complete building taxonomy for all digital access and literacy purposes e.g., goal setting, services needed, services provided.
- Train CHWs, patient navigators and peer supporters to work as digital navigators in health care.
- Support trained community-based digital navigators to work in health care.
- Encourage credentialing and allow reimbursement of digital navigators and for digital navigation services.
- Require reporting on digital access and literacy screening and on disparities in digital health use for all of the "covered populations" recognized in the Digital Equity Act. Include these as quality measures.

Integrate key themes into APM requirements [VB-2]

Population health and risk stratification are essential to improving health outcomes, healthcare quality and reducing costs. Stratification should be done by the DEA- covered population groups and geographies associated with access and adoption disparities: Rural/Urban/Suburban; historic redlining status and level of residential segregation in areas served by the health system or plan.

HHS policies to supplement CEHRT requirements [VB-8]

CEHRT requirements should ensure that individuals facing foundational digital access and literacy barriers participate in product development. They should also require that products operate in languages common in the population served by an institution subscribing to the services. Products should run on both IOS and Android systems and be backward compatible with low end and older devices. Products requiring compatible devices and internet connectivity should only be reimbursed if there are mechanisms in place to provide the devices and connectivity to users who need them.

Require that certified EHR vendors make sandbox versions of both institution-neutral and institution-specific sandbox version available for third party and community-based trainers to teach people to use portals.